



February 20, 2026

Department of Health and Human Services,  
Assistant Secretary for Technology Policy and ONC for Health Information Technology  
Attention: HHS Health Sector AI RFI  
Mary E. Switzer Building, Mail Stop: 7033A  
330 C Street SW, Washington, DC 20201

Subject: HHS RFI Response: Accelerating the Adoption and Use of AI as Part of Clinical Care

Dear Assistant Secretary and ONC Leadership,

MHK appreciates the opportunity to provide comments on the Request for Information (RFI) published on December 23, 2025, regarding strategies to accelerate the adoption and use of Artificial Intelligence (AI) in clinical care. We commend HHS, ASTP, and ONC for inviting stakeholder input across regulation, reimbursement, and research and development to responsibly scale AI tools that improve outcomes, reduce burden, and lower costs for patients and government programs.

MHK delivers solutions that support more than 90 million lives served by health plans and pharmacy benefit managers nationwide, in care management, utilization management (including prior authorization), pharmacy management, appeals and grievances, and population health. Our mission is to enable health plans to deliver efficient, compliant, and patient-centered care through advanced technology solutions. AI can transform care delivery, reduce administrative burden, and improve outcomes when implemented responsibly and transparently.

MHK supports HHS's vision to accelerate AI adoption in clinical care and has developed responsible AI enablement within our platform. The AI Agentic event framework MHK has designed and implemented internally has been recognized by our channel partner, Amazon Web Services (AWS), as a best-in-class approach compared to other solutions in the market. Within this framework, we are leveraging AI agents to streamline processes and reduce administrative burdens. Our approach prioritizes compliance, transparency, and interoperability to ensure trust and safety. We offer the following detailed responses to the RFI questions based on our experience designing, validating, and integrating AI into payer workflows. We also welcome the opportunity to meet, share our learnings, and partner with you.

Sincerely,

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## Detailed Responses to RFI Questions

**Question 1:** What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

- Despite progress through FHIR and related ONC initiatives, access to complete, high-quality, standards-based data across EHRs, payer systems, and ancillary sources remains inconsistent. AI systems rely on longitudinal, well-structured datasets, and fragmentation increases development cost, limits generalizability, and slows deployment. Model Context Protocol (MCP) integration can accelerate adoption by enabling secure access to clinical data across client environments without requiring data migration or format standardization.
- Fee-for-service reimbursement models do not always incentivize technologies to reduce utilization, shorten administrative cycle times, or lower the total cost of care. Therefore, AI tools that improve efficiency or prevent unnecessary services may lack a direct reimbursement pathway.
- Public concern regarding algorithmic bias, lack of transparency, and the risk of automation-driven denials, particularly in prior authorization, creates skepticism and slows adoption even where safeguards exist. **One approach is to avoid training proprietary AI models independently and instead** use industry-standard foundation models (Anthropic Claude, Google Gemini) and provide clinical context through Retrieval-Augmented Generation (RAG) via MCP-connected data sources and unstructured document processing. This can reduce the introduction of training data bias. The bias concerns can be limited to prompt injection (malicious input manipulation), cross-tenant data access, decision confidence fairness, and RAG context completeness (ensuring the right clinical evidence is retrieved for all patient types).
- Ambiguity in compliance frameworks, particularly around HIPAA and state-specific privacy regulations, can hinder adoption. Implementing comprehensive HIPAA Technical Safeguards (45 CFR § 164.312) could streamline adoption. This should include: Access Control with 3-tier capability token system (24-hour job tokens, 1-hour agent tokens with automatic expiration), Audit Controls with 365-day CloudWatch retention and 7-year archive capability, Integrity Controls with client-specific KMS encryption, Person/Entity Authentication via AWS Cognito OAuth2, and Transmission Security with TLS 1.3. Our audit framework generates 9 report types (daily/monthly/quarterly/annual/on demand), providing concrete evidence of compliance.

- Uncertainty regarding legal frameworks and governance, particularly relating to liability allocation, indemnification, and the documentation of human oversight, delays contracting and implementation processes, especially for AI applications within utilization management or clinical decision support integrated into electronic health records (EHRs). It is critical to have human-in-the-loop governance at multiple levels, including decision separation, confidence-based routing, progressive automation with safeguards, and administrative controls.

**Question 2:** What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care, and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

- HHS should prioritize payment models that reward AI-enabled reductions in administrative burden, improvements in the timeliness of care, and measurable improvements in outcomes. Introduce value-based incentives for AI-enabled solutions and documentation tools that demonstrate reduced administrative burden, costs, and improved outcomes. Additionally, HHS should simplify approval pathways for AI-driven utilization management tools, provided they maintain clinical oversight by licensed professionals.
- HHS policy should clearly distinguish between AI that assists clinicians or reviewers and AI that autonomously determines coverage or clinical actions. Existing CMS guidance stating that AI may support but not replace individualized medical necessity determinations should be codified.

**Question 3:** For non-medical devices, we understand that the use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (including liability, indemnification, privacy, and security). What novel legal and implementation issues exist, and what role, if any, should HHS play to help address them?

- We recommend establishing predictable regulatory guidance that balances innovation with patient safety. When AI informs but does not independently make clinical or utilization management decisions, existing legal frameworks do not clearly address:
  - Allocation of liability when AI recommendations are followed or overridden
  - Responsibility for performance drift, bias, or misclassification
  - Documentation standards demonstrating meaningful human review
- HHS could play a constructive role by:
  - Issuing guidance on minimum governance expectations for clinical AI, including human-in-the-loop workflows, audit trails, and documentation requirements.
  - Providing optional model contract language addressing indemnification and liability for AI-assisted clinical use cases.
  - Clarifying permissible uses and disclosures under HIPAA for AI training, monitoring, and safety auditing, including recommended de-identification practices.

**Question 4:** For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (*e.g.*, contracts, grants, cooperative agreements, and/or prize competitions)?

- Promising evaluation methods and metrics include: bias and fairness testing using parity metrics; robustness testing with documentation standards; post-deployment continuous performance surveillance of FHIR loads and connections; and payer-linked outcomes (appeal rates, overturn rates, adverse events) for AI in prior authorization and care coordination.
- HHS should support funding public-private development, such as shared datasets (clinical and claims), evaluation protocols, and independent measurement teams to prioritize tools, ambient documentation, prior authorization, and health care navigation. We recommend establishing grants for model registries and audit tools that use standardized metric schemas and reporting. HHS can also support competitions that incentivize reproducible processes that reduce bias and improve clinician trust, with performance-based awards.

**Question 5:** How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

- HHS should support a voluntary, risk-tiered assurance framework aligned with ONC certification and TEFCA. HHS should also recognize the distinction between AI platforms that train proprietary models and those that orchestrate industry foundation models.
- HHS should encourage third-party accreditation for prior authorization using AI and clinical documentation using AI (scribes), with audit trails that demonstrate human-in-the-loop governance, explainability, and outcomes (timeliness, overturn rates, adverse events).
- Codify payer expectations and guardrails for AI use in coverage decisions and ensure the architecture embeds key guardrails.
- Funding AI literacy and governance training for clinicians, medical directors, and utilization management professionals. Training curricula could incorporate anonymized examples to teach clinicians how to interpret AI confidence scores, recognize performance drift, and exercise effective oversight.

**Question 6:** Where have AI tools deployed in clinical care met or exceeded performance and cost expectations, and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights into quality, and help reduce costs?

- Ambient clinical documentation tools have met expectations by reducing clinician burnout, preserving workforce capacity, and improving documentation quality.
- AI deployment often lags expectations due to integration complexity, governance requirements, and validation burden.
- AI tools with High-Potential include prior authorization orchestration with real-time data retrieval and human override, population-level risk navigation integrating social drivers and benefits data, and explainable quality signal extraction beyond traditional Healthcare Effectiveness Data and Information Set (HEDIS) measures.

**Question 7:** Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

- Roles that are key influencers and decision makers among health plans include:
  - Chief Medical Officer/Medical Directors (clinical sponsorship, guardrails).
  - Chief Information Officers and Clinical Informatics (workflow integration, EHR alignment).

- Utilization Management/Case Management Leaders for payer-side adoption in prior authorization, concurrent review, and care coordination.
- Compliance/Privacy/Legal Officers.
- Primary administrative hurdles to the adoption of AI include
  - Interoperability friction, such as fragmented EHRs, a lack of standardized APIs, and mixed protocols.
  - Payment uncertainty, including the fact that fee-for-service rarely rewards deflationary technology, and coverage decisions could be slow.
  - Site-specific performance drift and bias audits, which could lead to validation and monitoring burdens.
  - Workforce readiness and change management are impacted by clinician trust, documentation standards, and governance.

**Question 8:** Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

- FHIR to enable plug and play across exchanges for clinical, claims, and prior authorization datasets.
- Claims and clinical linkage for outcomes and total costs supported by AI tools (LOS, readmissions, adverse events, equity impacts).
- Benchmarking tools such as national model registries with aligned metrics for pre- and post-deployment data.

**Question 9:** What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

- Some of the challenges that patients and caregivers wish to see addressed by the adoption and use of AI include:
  - Faster access to care by reducing prior authorization delays and having real-time decisions, as well as transparent reasons for determinations.
  - Fewer diagnostic delays because of AI triage and timely routing.
  - Increasing clinician presence: AI would reduce time spent on charting and increase time spent listening to ambient documentation.
- Some of the concerns that patients and caregivers wish to see addressed by the adoption and use of AI include:
  - Patients express concern about algorithmic bias and unequal denials, particularly in utilization management.
  - There is significant concern about batch denials caused by automation.
  - Sharing data over networks raises privacy and trust concerns.

**Question 10:** Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

- a. Are there published findings about the impact of adopted AI tools and their use in clinical care?
- b. How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?

- The specific areas of AI research that HHS should prioritize include:
  - Implementation science for AI-enabled prior authorization
  - Real-world performance drift and monitoring methodologies
  - Total cost of ownership and payment models for deflationary technologies
  - Explainable quality measures linked to clinical outcomes

**Conclusion:**

MHK is committed to advancing responsible AI adoption in clinical care, and we applaud the Administration's efforts to accelerate its use across the health care system. We believe collaboration between HHS, health plans, and technology partners is essential to achieving better outcomes, reducing costs, and improving patient experience. The AI agentic event framework we have built internally has been recognized by our channel partner, Amazon Web Services, as a best-in-class approach compared with other solutions in the market. Based on our experience designing, validating, and integrating AI into payer workflows, we would welcome the opportunity to meet, share our learnings, and partner with you.

For reference, some of MHK's current AI initiatives include:

- Automated Prior Authorization: Leveraging AI to reduce turnaround times while maintaining compliance.
- AI Agentic Enablement: Extracting insights from clinical notes to support decision-making.
- Predictive Analytics for Care Management: Identifying high-risk members for early intervention.
- Member Engagement Tools: AI-driven outreach tools to improve adherence and satisfaction.